

Started on	Sunday, 12 February 2023, 8:24 PM
State	Finished
Completed on	Sunday, 12 February 2023, 8:24 PM
Time taken	17 secs
Marks	0.00/113.00
Grade	0.00 out of 100.00



Question 1

Not answered

Marked out of 5.00

ADHD treatments & side effects

Identify the most likely drug responsible for each of the following side effects while treating children with ADHD

Mild growth slowing especially during first two years	Choose... ▾
Monitoring of liver function tests is required	Choose... ▾
Exacerbation of tics	Choose... ▾
ECG monitoring required in children	Choose... ▾
Hallucinations such as skin crawling and visions	Choose... ▾

Your answer is incorrect.

Explanation:

- 1) Mild growth slowing has been reported with methylphenidate especially during first two years, and it is vital to monitor height & weight regularly.
- 2) Atomoxetine could cause gastrointestinal side effects and impairment of liver function tests. Therefore, monitoring of LFTs is recommended.
- 3) Exacerbation of tics has been reported with stimulant medication such as methylphenidate.
- 4) Amitriptyline could cause cardiac conduction abnormalities, and ECG monitoring is recommended.
- 5) Hallucinations - both tactile and visual - have been reported with methylphenidate.

The correct answer is: Mild growth slowing especially during first two years → Methylphenidate, Monitoring of liver function tests is required → Atomoxetine, Exacerbation of tics → Methylphenidate, ECG monitoring required in children → Amitriptyline, Hallucinations such as skin crawling and visions → Methylphenidate

Question 2

Not answered

Marked out of 5.00

Etiology of childhood disorders

Identify the most important etiological factors associated with each of the following diagnoses.

School refusal	Choose... ▾
Depressive episode	Choose... ▾
Attention deficit hyperactivity disorder	Choose... ▾
Conduct disorder	Choose... ▾
Anorexia nervosa	Choose... ▾

Your answer is incorrect.

Explanation:

- 1) Being the youngest child in the family is associated with school refusal.
- 2) Adolescent depression is associated with increased cortisol levels.
- 3) Maternal smoking during pregnancy increases the risk of development of ADHD in children.
- 4) The presence of callous traits in parents is a risk factor for development of conduct disorder in children. Anorexia is much widely found in the developed countries (western world), suggesting a critical role for Western social ideals of thinness.

The correct answer is: School refusal → Youngest child in family, Depressive episode → Increased cortisol levels, Attention deficit hyperactivity disorder → Maternal smoking during pregnancy, Conduct disorder → Callous traits in parents, Anorexia nervosa → Living in the western world

Question 3

Not answered

Marked out of 3.00

Rating scales in children (1)

Select one rating scale for use in each of the following situations;

This scale is useful to assess the severity of obsessive-compulsive disorder in a 12-year-old boy

Choose...

This scale is helpful to measure severity of depressive symptoms in a 13-year-old girl

Choose...

This scale is widely used to obtain information from school teachers on ADHD symptoms

Choose...

Your answer is incorrect.

Explanation:

- 1) CY-BOCS is the Paediatric version of Y-BOCS (Yale-Brown obsessive-compulsive scale).
- 2) Children's depression inventory is a scale that is useful to measure the severity of depressive symptoms between 7 to 17 years. Beck Depression Inventory could be used for children only after the age of 14.
- 3) Connor's questionnaire is widely used to obtain information from schoolteachers on ADHD symptoms. There are also the parent and adolescent versions.

The correct answer is: This scale is useful to assess the severity of obsessive-compulsive disorder in a 12-year-old boy → Yale-Brown scale for children, This scale is helpful to measure severity of depressive symptoms in a 13-year-old girl → Children's depression inventory, This scale is widely used to obtain information from school teachers on ADHD symptoms → Connor's questionnaire

Question 4

Not answered

Marked out of 5.00

Male: Female ratios

Identify the male: female ratio for each of the childhood psychiatric conditions listed below

Bulimia nervosa

Choose... ▾

Asperger's syndrome

Choose... ▾

Attention deficit hyperactivity disorder in community surveys

Choose... ▾

Pre-pubertal depression

Choose... ▾

Tourette's syndrome

Choose... ▾

Your answer is incorrect.

Explanation:

- 1) Bulimia nervosa is seen more often in females with a male: female ratio of 1:10.
- 2) Asperger's syndrome: The ratio of males to females with AS is currently estimated to be 9:1. On an average, boys are referred nine times more often for diagnostic assessment itself (Wagner, 2006).
- 3) ADHD is seen more often in boys with a male predominance of 3:1.
- 4) Prepubescent depression: The prevalence is up to 3% with 1:1 gender ratio. Adolescent depression: The prevalence is approximately 8% for males and 14% for females.
- 5) The mean age of onset of Tourette's syndrome is seven years and male: female ratio is 3:1.

The correct answer is: Bulimia nervosa → 1:10, Asperger's syndrome → 9:1, Attention deficit hyperactivity disorder in community surveys → 3:1, Pre-pubertal depression → 1:1, Tourette's syndrome → 3:1

Question 5

Not answered

Marked out of 5.00

Treatment of nocturnal enuresis

Identify the treatment options for nocturnal enuresis, using the descriptions given below:

The most effective treatments for nocturnal enuresis

Choose...

First step in treatment plan of nocturnal enuresis

Choose...

This compound is available as an intranasal spray and has shown initial success in the treatment of nocturnal enuresis in children

Choose...

This tricyclic antidepressant has been approved for use in treating childhood enuresis, primarily on a short-term basis.

Choose...

This norepinephrine reuptake inhibitor with a non cardiotoxic side effect profile is a safer alternative in the treatment of enuresis in children

Choose...

Your answer is incorrect.

Explanation:

- 1) Classic conditioning with the bell and pad apparatus is the most effective treatment for nocturnal enuresis, with dryness resulting in more than 60 percent of cases.
- 2) The first step in any treatment plan is to review appropriate toilet training.
- 3) Desmopressin, an antidiuretic compound that is available as an intranasal spray, has shown some initial success in reducing enuresis.
- 4) Imipramine is efficacious and used on a short-term basis. It has been approved for use in treating childhood enuresis, primarily on a short-term basis.
- 5) Reboxetine is a norepinephrine reuptake inhibitor. It has a non-cardiotoxic side effect profile. It has recently been investigated as a safer alternative to imipramine in the treatment of childhood enuresis.

The correct answer is: The most effective treatments for nocturnal enuresis → Classic conditioning with the bell and pad apparatus, First step in treatment plan of nocturnal enuresis → Appropriate toilet training, This compound is available as an intranasal spray and has shown initial success in the treatment of nocturnal enuresis in children → Desmopressin, This tricyclic antidepressant has been approved for use in treating childhood enuresis, primarily on a short-term basis. → Imipramine, This norepinephrine reuptake inhibitor with a non cardiotoxic side effect profile is a safer alternative in the treatment of enuresis in children → Reboxetine

Question 6

Not answered

Marked out of 2.00

Childhood & developmental disorders (2)

For each of the following vignette below choose one correct answer from the list:

A 12-year-old boy has poor attendance record at the school. His academic achievement is weak. He frequently spends the day out with his friends in the cinema or shopping centre.

Choose...

A 14-year-old girl with anhedonia, poor appetite, insomnia, poor concentration and lacking the motivation to do anything.

Choose...

Your answer is incorrect.

Explanation:

1) In truancy, there is a poor academic achievement and poor attendance record at the school. The presence of anti-social symptoms is more suggestive of truancy.

2) Major depression is the most severe condition in adolescents, with either sad or irritable mood and or anhedonia. There are at least five other symptoms, such as social withdrawal, worthlessness, guilt, suicidal thoughts or behaviour, sleep increase or decrease, decreased motivation and concentration, and increased or decreased appetite.

The correct answer is: A 12-year-old boy has poor attendance record at the school. His academic achievement is weak. He frequently spends the day out with his friends in the cinema or shopping centre. → Truancy, A 14-year-old girl with anhedonia, poor appetite, insomnia, poor concentration and lacking the motivation to do anything. → Depressive disorder

Question 7

Not answered

Marked out of 3.00

Rating scales in children (2)

Select one rating scale to use in each of the following situations

This scale is widely used to obtain information from school teachers on ADHD symptoms

Choose...

A structured interview mainly used in research to provide a thorough assessment of individuals suspected of having autism

Choose...

This scale can be helpful in the evaluation of a child with multiple behavioural problems, especially if the child presents with different symptoms in different settings such as at school and home

Choose...

Your answer is incorrect.

Explanation:

- 1) Connor's questionnaire is widely used to obtain information from schoolteachers on ADHD symptoms. There are also the parent and adolescent versions.
- 2) Autism diagnostic interview- Revised: It is a structured interview mainly used in research to obtain a thorough assessment of individuals suspected of having autism. It is useful for formal diagnosis as well as treatment and educational planning. It is composed of 93 items focusing on 3 areas; Language & communication, reciprocal social interactions, restricted & stereotyped interests and behaviours.
- 3) The Child Behavior Checklist (CBCL) assess a broad range of symptoms that relate to academic and social competence of children aged 4 through 16. They are reported by their parents or others (e.g., teachers) who know the child well. The checklist is composed of 113 items in a Likert scale. This scale can be helpful in the evaluation of a child with multiple behavioural problems, especially if the child presents with different symptoms in different settings such as at school and home. There is a parent and a teacher version so that the reports of these two observers may be compared.

The correct answer is: This scale is widely used to obtain information from school teachers on ADHD symptoms → Connor's questionnaire, A structured interview mainly used in research to provide a thorough assessment of individuals suspected of having autism → Autism

diagnostic interview-revised, This scale can be helpful in the evaluation of a child with multiple behavioural problems, especially if the child presents with different symptoms in different settings such as at school and home → Child behaviour checklist (CBCL)



Question 8

Not answered

Marked out of 5.00

Treatment options in child psychiatry (2)

Identify the best modes of treatment for each of the following clinical situations

A 13-year-old boy has extreme contamination fears and spends more than 2 hours of daily compulsive hand washing. He has difficulty going to school due to this behaviour

Choose...

A 15-year-old boy has not been to his school for last few weeks as he has been disturbed by derogatory auditory hallucinations. He has been hearing voices in the third person for two months and is receiving messages from the television.

Choose...

A 16-year-old girl has been admitted to hospital recently. She became depressed and attempted to harm herself by taking an overdose

Choose...

A 9-year-old girl became disobedient and defiant shortly after her father married a woman with two children

Choose...

A 9-year-old girl is ashamed of her bedwetting and will not attend any sleepover parties

Choose...

Your answer is incorrect.

Explanation:

1) The diagnosis is OCD. Sertraline is licensed in the UK for the treatment of OCD in young people. The Paediatric OCD Treatment Study (POTS) is a recent multi-site National Institute of Health-funded investigation of sertraline and cognitive-behavioural therapy (response prevention) each alone and in combination for the treatment of childhood-onset OCD. The POTS study revealed that the combination was superior to either treatment alone.

2) The diagnosis is early onset schizophrenia and treatment should be one of the second-generation antipsychotics like Risperidone (low dose).

3) The diagnosis is childhood depression, and combination treatments should be tried. Combination treatment should be considered in all cases (i.e. psychotherapy + medication). The final CBT efficacy trial in a diagnosed sample is the Treatment of Adolescents with Depression Study (TADS). TADS is the only published study to compare CBT, fluoxetine, their combination, and placebo. Combined treatment was superior to fluoxetine alone with regard to remission (37 vs. 20%).

4) The diagnosis is oppositional defiant disorder. The girl is expressing her discomfort about a significant change in the family's functioning. Family therapy is a useful modality to understand the triggers, responses and meanings of these behaviours to the family and to the child. It is likely that the behaviours will diminish when the child is given a forum in which to express her discomfort.

5) The diagnosis is nocturnal enuresis. Although desmopressin nasal spray is effective in some children with enuresis, it may be useful only in the short-term and the most effective treatment is a behavioural approach using methods such as the bell and pad conditioning treatment. (Kaplan & Sadock review in psychiatry-pg 412)

The correct answer is: A 13-year-old boy has extreme contamination fears and spends more than 2 hours of daily compulsive hand washing. He has difficulty going to school due to this behaviour → Sertraline plus response prevention, A 15-year-old boy has not been to his school for last few weeks as he has been disturbed by derogatory auditory hallucinations. He has been hearing voices in the third person for two months and is receiving messages from the television. → Risperidone, A 16-year-old girl has been admitted to hospital recently. She became depressed and attempted to harm herself by taking an overdose → Fluoxetine plus CBT, A 9-year-old girl became disobedient and defiant shortly after her father married a woman with two children → Family therapy, A 9-year-old girl is ashamed of her bedwetting and will not attend any sleepover parties → Desmopressin nasal spray plus behavioral conditioning



Question 9

Not answered

Marked out of 4.00

Childhood Disorders

Choose the correct diagnosis for each of the following situations

An 8-year-old girl is restless, not sleeping well and has frequent stomach aches. She is also missing school frequently

Choose...

A 7-year-old girl cannot speak at the school when called upon by teachers. However, she whispers' in her friend's ear to communicate

Choose...

A 7-year-old girl is attending ballet dance class in her school. However, she is extremely fearful to perform in front of the public and would cause an embarrassment. She is missing her school frequently.

Choose...

A 10-year-old boy was caught in a fire accident four months ago. He has difficulty going to sleep, recurrent recollections of the event and has nightmares. He has poor concentration, and there is a decline in his school academic performance.

Choose...

Your answer is incorrect.

Explanation:

- 1) The diagnosis is generalized anxiety disorder. The symptoms in children include somatic symptoms (such as headache, stomach pains, rapid heartbeat, shortness of breath), nail biting, hair pulling and school refusal.
- 2) The diagnosis is selective mutism. It is a disorder characterized by a persistent failure to speak in specific settings (school) despite the full use of language at home or with family, may be found in younger children with social phobia. A child with selective mutism may remain completely silent or near silent, in some cases whispering instead of speaking out loud.
- 3) The diagnosis is a social phobia, which is characterized by marked fear of one or more social, or performance-related situations where the person is exposed to scrutiny and in which embarrassment may occur. The exposure to social situations usually causes an anxiety reaction that is recognized as inappropriate. Thus, situations are either avoided or endured with discomfort. In severe cases, it may lead to school refusal in children.

4) Posttraumatic stress disorder (PTSD) is characterized by a set of symptoms such as intrusive thoughts about a traumatic event, persistent avoidance, and hyperarousal (poor sleep, poor concentration leading to decline in performance at school)in response to exposure to one or more traumatic events. Unlike most anxiety disorders, PTSD involves reactions to memories of past events rather than the anticipation of an impending threat.

The correct answer is: An 8-year-old girl is restless, not sleeping well and has frequent stomach aches. She is also missing school frequently → Generalised anxiety disorder, A 7-year-old girl cannot speak at the school when called upon by teachers. However, she whispers' in her friend's ear to communicate → Selective mutism, A 7-year-old girl is attending ballet dance class in her school. However, she is extremely fearful to perform in front of the public and would cause an embarrassment. She is missing her school frequently. → Social phobia, A 10-year-old boy was caught in a fire accident four months ago. He has difficulty going to sleep, recurrent recollections of the event and has nightmares. He has poor concentration, and there is a decline in his school academic performance. → PTSD



Question 10

Not answered

Marked out of 7.00

ADHD treatment options

For each of the clinical situations related to ADHD described below, identify the correct mode of treatments from the list below

1. A 9-year-old school boy who needs once daily dosing (TWO options)  
  
2. A 7-year-old boy has been diagnosed with ADHD. His father is keen to know the name of the drug that is usually the first line treatment for ADHD and is also well tolerated  
3. A 7-year-old boy has been treated for ADHD for more than two years. Of late he grimaces and makes funny movements with his arms and shoulders, which is embarrassing for his parents (Choose TWO)  
  
4. A 9-year-old girl requires treatment for ADHD symptoms. Of late she is also low in mood, not mixing with peer group, can't sleep and has no appetite  
5. An 8-year-old boy has been treated for ADHD. Of late, he is becoming aggressive at home and school  

Explanation:

1) Methylphenidate is now available in XL preparation. It could be administered once a day, preferably in the mornings. Atomoxetine can be given once daily and can be given at any time of day.

2) Methylphenidate is the first line treatment for ADHD, which is well tolerated and specifically treat ADHD core symptoms.

3) This boy has developed tics along with ADHD, possibly as a side effect of his medications. Although a trial of atomoxetine if tics worsen with stimulants, cases of tics with atomoxetine have been reported. Therefore, clonidine would be a good choice.

4) This girl is suffering from depression and ADHD. Tricyclics is helpful to treat both ADHD and comorbid depression/anxiety.

5) For children with ADHD on methylphenidate and has aggressive behaviour, Atomoxetine is unlikely to be beneficial. Consider behavioural therapy before treating them on antipsychotics.

Correct Answer is : 1. Atomoxetine, Methylphenidate sustained release 2. Methylphenidate 3. Atomoxetine, Clonidine 4. Amitriptyline 5. Behavioural Therapy



Question 11

Not answered

Marked out of 5.00

Conduct disorder management

With the descriptions given below, identify the correct mode of treatments for conduct disorder

1. Prevention strategies for conduct disorder (Choose TWO)



2. In this therapy, assessing and promoting the strengths of the young person and the system is paramount.



3. This therapy focuses on improving communication between parent and young person. It focuses on reducing inter-parental inconsistency, negotiating rules and the sanctions to be applied for breaking them.



4. This therapy in teenagers typically includes social skills training and anger management.

**Explanation:**

1) Preschool child development programmes aims at identifying parents and families at risk and instituting home visits and support. Community programmes aim to identify children and adolescents through their involvement with social agencies and institute interventions such as enhanced recreational programmes, parent training and adult mentoring of youth. These strategies can help to prevent at least some of the conduct disorder cases from emerging.

2) Multisystemic therapy is one of the best-developed treatments of conduct disorder. In this treatment, the young person's and family's needs are assessed in their context at home and in their relationships with other systems such as school and peers. Following the assessment, proven methods of intervention is used to address difficulties and promote strengths. In this therapy, assessing and developing the strengths of the young person and the system is paramount.

3) Functional family therapy is one of the best-known interventions for serious antisocial behaviour. Functional family therapy addresses family processes and focuses on improving communication between parent and young person. It aims to reduce interparental inconsistency, tightening up on supervision and monitoring. It helps to negotiate rules and the sanctions to be applied for breaking them.

4) CBT for conduct problems in children and adolescents typically includes social skills training and anger management. The most common targets are aggressive behaviour, social interactions, self-evaluation and emotional dysregulation.

Correct Answer is : 1. Preschool child development programme, Community programmes 2. Multimodal systemic therapy 3. Functional family therapy 4. CBT



Question 12

Not answered

Marked out of 5.00

Treatment options in CAMHS

From the list below select the ONE most likely course of action for each of the following situations:

1. A woman brings her 11-year old son to your clinic, saying that he is very anxious. He takes an exceedingly long time to get ready in the morning. An interview with the child reveals that he has 'silly' thoughts that come into his head. He says he recognises them as his own, but he dislikes them and has to do specific things to 'send them away'. (Choose TWO)

2. A 12 yr old boy with low mood, loss of interest, weight loss.

3. A 17-year-old boy with autism has frequent aggressive behaviour and has been in trouble with the police. He has been recently banned from his home. He is reported to be aggressive almost all of the time.

4. A 4-year-old girl, who is aggressive towards her mother and has punched her in the stomach on one occasion, is difficult and refuses to do what she is told.

Explanation:

1) The diagnosis is obsessive-compulsive disorder. Sertraline is licenced in the UK for the treatment of childhood OCD; fluoxetine can also be used.

2) The diagnosis is childhood depression. Fluoxetine has the best evidence base for the treatment of depression in children.

3) Low dose risperidone is effective in the management of aggressive and challenging behaviour in some patients with autism.

4) Parental skills training should be tried in young children. It helps parents manage specific problematic or situations. The aim is to help parents avoid ineffective parental responses, by learning effective ways of managing children's behaviour.

Correct Answer is : 1. Sertraline , Fluoxetine 2. Fluoxetine 3. Risperidone 4. Parental skills training



Question 13

Not answered

Marked out of 7.00

Tic disorders

Identify 2 features for each of the following conditions;

1. PANDAS Syndrome



2. Tourette's syndrome



3. Transient tic disorder



4. Chronic tic disorder

**Explanation:**

1) PANDAS syndrome: Paediatric autoimmune neurological disorder associated with streptococcus. An autoimmune syndrome associated with OCD and tic disorder. Some children appear to develop obsessive-compulsive symptoms associated with beta haemolytic streptococcal infection and this presentation represents a small proportion of OCD cases in this population. This association has led to the studies of immune responses in OCD. Specific characteristics include onset before puberty, episodic or 'sawtooth' course and choreiform movements.

2) Tourette's syndrome is characterised by multiple motor and one or more vocal tics, causing distress and impaired function. The tics occur many times a day (usually in bouts) nearly every day or intermittently throughout a period of more than one year. During this period, there was never a tic-free period of more than three consecutive months. Facial tics: eye blinks or head jerks are often the initial symptoms, but tics involving the neck, shoulders, and upper extremities are also common.

3) Transient tic disorder: Single or multiple motor and/ or vocal tics (sudden, rapid, recurrent, nonrhythmic, stereotyped motor movements or vocalisations). Usually, face, nose, throat are involved (grimaces, blinks, frowns, grunts, throat clearing, sniffs). The tics occur many times a day, nearly every day for at least four weeks, but for no longer than 12 consecutive months.

4) Chronic Tic disorder persist for more than one year and often shows relapses and remissions throughout childhood. Maybe one or several tics are simultaneously present. Motor or Vocal tics but not both. Over time, one form fades to be replaced by another.

Correct Answer is : 1. Autoimmune neurological disorder associated with OCD and tic disorder, Exacerbation of symptoms of beta haemolytic streptococcal infection 2. Tics persist for more than 12 months, Multiple motor and one or more vocal tics present 3. Multiple motor and one or more vocal tics present, Tics last less than 12 months 4. Tics persist for more than 12 months, Motor or Vocal tics but not both



Question 14

Not answered

Marked out of 9.00

Depression in children and adolescents

Identify 3 characteristic features for each of the following conditions:

1. Depression in young children



2. Depression in older children



3. Depression in adolescents



Explanation: The clinical features of depression can vary according to their ages.

1) Young children (0 to 5 year): Poor feeding, failure to thrive, separation anxiety, tantrums, irritability, aggression, hyperactivity, regressed behaviour (enuresis, soiling, etc.).

2) Older children (6-10 years)- somatisation, school refusal, poor school performance, acting out.

3) Adolescents- Low self-esteem, melancholia, suicidal acts, substance misuse, psychotic symptoms.

Correct Answer is : 1. Poor feeding, Failure to thrive, Tantrums 2. Somatisation, Poor school performance, School refusal 3. Low self-esteem, Suicidal acts, Substance misuse

Question 15

Not answered

Marked out of 11.00

Pervasive developmental disorders

Identify 3 essential features of each of the following conditions

1. Autism

 ✖ ✖ ✖

2. Asperger's syndrome

 ✖ ✖ ✖

3. Rett's syndrome

 ✖ ✖ ✖

4. Hellers' syndrome

 ✖ ✖ ✖**Explanation:**

1) Autistic Disorder is historically known as infantile autism, childhood Autism, or Kanner's autism. It is characterized by symptoms from each of the following three categories: Qualitative impairment in social interaction, Impairment in communication [language delay is most common cause for initial referral], Restricted repetitive and stereotyped patterns of behaviour or interests. 70% have mild to moderate learning disability.

2) Asperger's syndrome is characterised by severe persistent impairment in social interactions, repetitive behaviour patterns, and restricted interests. IQ and language are normal. Unlike autistic disorder, in Asperger's syndrome no significant delays occur in language, cognitive development, or age-appropriate self-help skills.

3) Rett's syndrome is an X linked dominant disorder of arrested Neuro-development, which affects almost exclusively females. Head circumference is normal at birth. Developmental milestones are unremarkable in early life. Between the ages of 6 months to 2 years, head growth begins to decelerate and produces microcephaly. Other signs often include the loss of purposeful hand movements, which are replaced by stereotypic motions, such as hand wringing; the loss of previously acquired speech; psychomotor retardation; and ataxia. Other stereotypical hand movements may occur, such as licking or biting the fingers and tapping or slapping.

4) Childhood disintegrative disorder (CDD) is also called Heller's syndrome and disintegrative psychosis. It is characterised by a marked regression in several areas of functioning after at least two years of apparently normal development. There is normal development for 2-3 years, followed by a loss of acquired motor, language, and social skills between ages three and four years. Stereotypies and compulsions are common.

Correct Answer is : 1. Severe persistent impairment in social interactions, Repetitive behavioural patterns and restricted interests, 70% have mild to moderate learning disability 2. IQ and language are normal, Severe persistent impairment in social interactions, Repetitive behavioural patterns and restricted interests 3. Affects almost exclusively females, Abnormal hand movements, First six months of life are normal 4. Normal development for 2-3 years, Disintegrative psychosis, Loss of acquired skills between ages 3-4



Question 16

Not answered

Marked out of 8.00

Language and learning disorders

Identify 2 important features for each of the following diagnoses

1. Reading disorder (dyslexia)



2. Disorders of written expression



3. Mathematics disorder



4. Stuttering

**Explanation:**

1) Reading disorder (dyslexia) is characterised by a deficit in phonological processing skills and an impaired ability to recognise words, slow and inaccurate reading, and poor comprehension. Male predominance is noted [4:1].

2) The disorder of written expression often coexists with dyslexia. It usually manifests as difficulties with spelling, syntax, grammar, and composition. Common features of the disorder of written expression are spelling errors, grammatical errors, punctuation errors, poor paragraph organisation, and poor handwriting. It occurs in 2-8% of school-age children with a 3:1 male predominance.

3) Mathematics disorder: Epidemiological studies have indicated that up to 6 percent of school-age children have some difficulty with mathematics. It is reported more commonly in females. Often associated with visuospatial deficits and attributed to right parietal dysfunction. The standard features of mathematics disorder include difficulty with various components of mathematics. It includes learning number names, remembering the signs for addition and subtraction, learning multiplication tables, translating word problems into computations, and doing calculations at the expected pace.

4) Stuttering is a condition in which the normal flow of speech is disrupted by involuntary speech motor events. Usually, struggle with initial syllables. Male: female ratio is 3:1. Management includes speech therapy.

Correct Answer is : 1. Deficit in phonological processing skills, Male predominance 2. Difficulties with spelling, syntax, grammar and comprehension, Male predominance 3. Female predominance, Often associated with visuospatial deficits 4. Treatment includes speech therapy, Usually, struggle with initial syllables



Question 17

Not answered

Marked out of 8.00

Child abuse & alerting signs

Identify the alerting signs most likely to be seen in children for the possible types of abuse given below

1. Signs of possible sexual abuse ✖ ✖2. Signs of possible emotional abuse ✖ ✖3. Signs of possible physical neglect ✖ ✖4. Signs of possible physical abuse ✖ ✖**Explanation:**

1) Discomfort and difficulty walking or sitting, stained underwear is a physical sign of possible sexual abuse. Some important signs include; Bruises, scratches, or other marks on the thighs or genital area. Some may have itching, soreness, discharge, unexplained bleeding from the rectum, vagina, or penis. Pain on passing urine, recurrent urinary tract infection, recurrent vaginal infection, unexplained pregnancy are other possible signs. Lack of trust in adults and fear of a particular individual is a behavioural sign of possible sexual abuse.

2) Low self-esteem, continual self-deprecation, Sudden speech disorder, Rocking, head-banging, or other neurotic behaviour are signs of possible emotional abuse.

3) Signs of possible physical neglect include constant hunger, poor personal hygiene, constant tiredness, a poor state of clothing, and untreated medical problems. A frequent lateness or non-attendance at school is also possible.

4) The signs of possible physical abuse include unexplained injuries, bruises or burns especially if recurrent. When enquired improbable excuses are given for injuries and also refuse to discuss injuries. There may be untreated injuries or delay in reporting them and history of receiving excessive physical punishment.

Correct Answer is : 1. Discomfort and difficulty walking or sitting , Lack of trust in adults 2. Low self-esteem, continual self-deprecation, Sudden speech disorder 3. Poor personal hygiene, Untreated medical problems 4. Untreated injuries or delay in reporting them, Recurrent unexplained injuries or burns



Question 18

Not answered

Marked out of 9.00

Parenting styles

For each of the following parenting styles identify the characteristics from the list provided

1. Authoritarian parenting style (Choose THREE)



2. Permissive parenting style (Choose THREE)



3. Authoritative parenting style (Choose THREE)

**Explanation:**

1) The features of authoritarian parenting style include detached and unaffectionate parents with power being imposed resulting in an autocratic situation that yields compliant but withdrawn and dependent children.

2) The features of permissive parenting style include indulgent and affectionate parents, but may be associated with immature children who lack purpose, self-control and self-direction, vulnerable to increased risk of future drug or alcohol abuse.

3) The features of authoritative parenting style include parents who use authority and household rules/limits with some explanation, attends to child's view and grant some responsibility to child, yields children who are independent, assertive and friendly.

Correct Answer is : 1. Detached and unaffectionate, Power imposed and Autocratic, Yields compliant but withdrawn and dependent children
2. Indulgent and affectionate, Yields immature children who lack purpose, self-control and self-direction, Vulnerable to increased risk of future drug or alcohol abuse
3. Uses parental authority and household rules/limits with some explanation, Attends to child's view and grant some responsibility to child, Yields children who are independent, assertive and friendly



Question 19

Not answered

Marked out of 7.00

Investigations in child psychiatry

For each of the medication below choose the most appropriate biochemical or physiological investigations required to monitor for adverse effects during routine treatment

1. Methylphenidate (2 options) ❌

❌

2. Sodium Valproate (3 options) ❌

❌ ❌

3. Atomoxetine (2 options) ❌

❌

Explanation:

Height, weight, blood pressure, heart rate monitoring is recommended for methylphenidate (initially 3 monthly, then 6 monthly).

In December 2012, MHRA issued a note highlighting that Atomoxetine can cause clinically relevant increases in blood pressure or heart rate, or both, in a small proportion of patients and suggested monitoring heart rate and BP. Atomoxetine rarely causes liver damage, so routine monitoring of LFT is not warranted. The MHRA also advises that patients on Atomoxetine should be monitored for signs of depression, suicidal thoughts or suicidal behaviour. Monitoring of height and weight is also recommended, initially three monthly and then six monthly.

For valproate, 6-monthly monitoring of LFT, full blood count and height/weight are recommended (CG38 NICE).

Correct Answer is : 1. Blood pressure and heart rate , Height and weight 2. Full Blood Count, Height and weight , Liver function tests 3. Blood pressure and heart rate , Height and weight



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